PC-06 Pediatric Wide-Complex Tachycardia

CLAIBORNE COUNTY EMS | MEDICAL-DIRECTOR APPROVED | SOURCE PDF | OCTOBER 26, 2026

Scope: Pediatric tachycardia with QRS greater than 0.09 seconds and suspected VT, SVT with aberrancy, or polymorphic tachycardia. Treat as potentially ventricular until proven otherwise.

Field flow

1. Assess	Perfusion, mental status, BP, respiratory status, QRS pattern, cardiac history, and reversible causes.
2. Compromise?	Hypotension, acute altered mental status, shock, or sudden collapse: prepare synchronized cardioversion immediately.
3. Cardioversion	0.5-1 J/kg; if unsuccessful, increase to 2 J/kg. Confirm sync mode before every shock; sedate only if it will not delay treatment.
4. Stable rhythm	Obtain early Medical Control/expert consultation. Adenosine may be considered only for a regular, monomorphic rhythm.
5. Reassess	Continuous monitoring, rhythm strips, cause-directed treatment, and early pediatric-capable destination notification.

Provider-level actions

Level	Approved actions
EMT	Oxygenation as indicated, identify compromise, obtain rhythm strip when available, and prepare for synchronized cardioversion.
AEMT	All EMT care plus IV/IO and supportive care; do not delay cardioversion or transport for repeated access attempts.
Paramedic	All prior care plus synchronized cardioversion for compromise and early Medical Control/expert consultation for stable wide-complex tachycardia.

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SAFETY, MEDICATION, AND LINKED-PATHWAY REFERENCE

Safety and escalation

• Do not delay synchronized cardioversion for sedation, 12-lead acquisition, or repeated IV attempts.

• Do not give adenosine for unstable, irregular, or polymorphic wide-complex tachycardia.

• If pulseless, begin CPR and use PC-04; for VF/pulseless VT use PC-07.

• Assess hypoxia, electrolyte/metabolic disturbance, toxin/stimulant exposure, myocarditis/cardiomyopathy, and congenital disease.

Medication reference

Medication	Approved dose / safeguard
Adenosine	0.1 mg/kg rapid IV/IO push with immediate saline flush; max 6 mg. Repeat 0.2 mg/kg; max 12 mg. Only for stable, regular, monomorphic rhythm with Medical Control/expert consultation.
Midazolam	0.1-0.2 mg/kg IV/IO/IN for cardioversion; max single 2 mg, max total 5 mg. Do not delay cardioversion; verify against pediatric medication reference.
Antiarrhythmic	Agent/dose determined by Medical Control. Do not administer amiodarone and procainamide together.

Linked pathways

AR-05 Pediatric Airway; PC-04 Pulseless Arrest; PC-05 Narrow-Complex Tachycardia; PC-07 VF / Pulseless VT; Medication Reference.

Source control

• This controlled direct source PDF replaces the prior source file and matches approved Claiborne EMS app content released October 26, 2026.

• Use current Claiborne standing orders, formulary, equipment authorization, and Medical Control direction whenever they govern a case-specific decision.